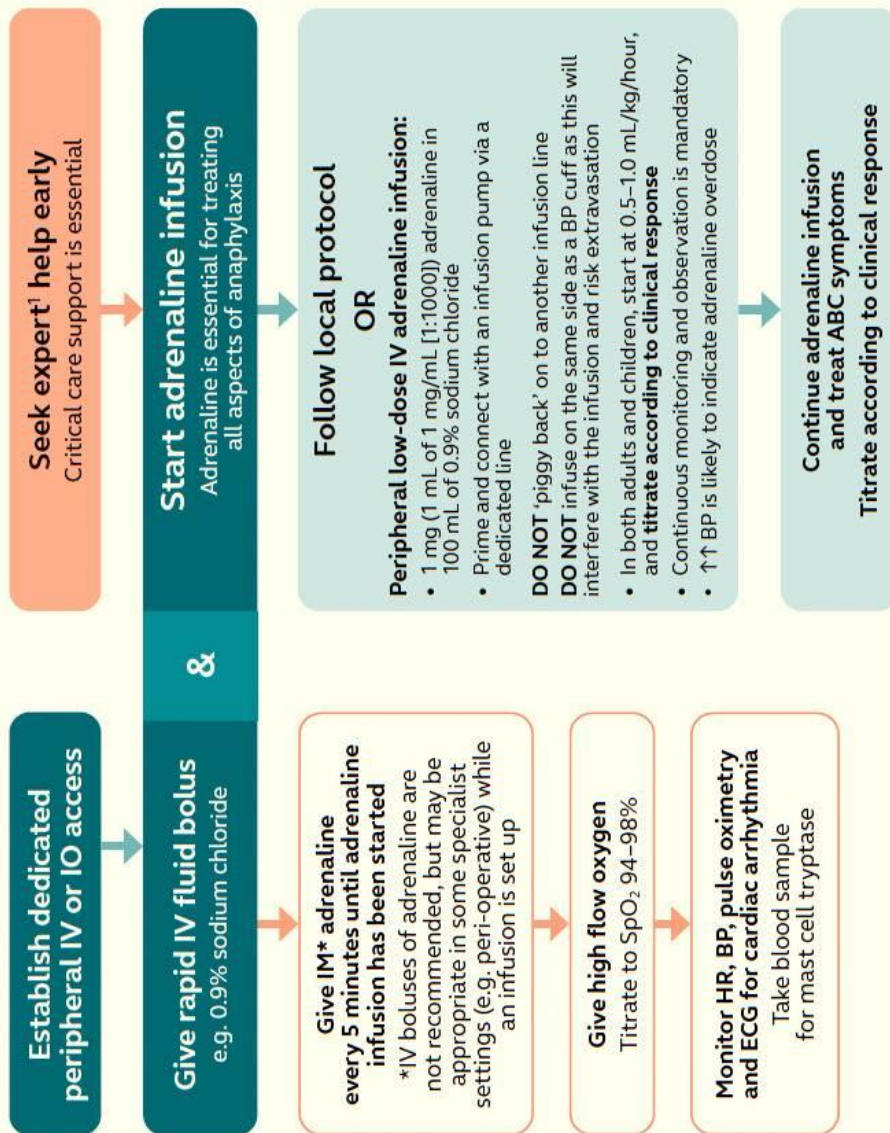




Refractory anaphylaxis

No improvement in respiratory or cardiovascular symptoms despite 2 appropriate doses of intramuscular adrenaline



¹Intravenous adrenaline for anaphylaxis to be given only by experienced specialists in an appropriate setting.

A = Airway
Partial upper airway obstruction/stridor:
Nebulised adrenaline (5mL of 1mg/mL)
Total upper airway obstruction:
Expert help needed, follow difficult airway algorithm

B = Breathing
Oxygenation is more important than intubation
If apnoeic:
• Bag mask ventilation
• Consider tracheal intubation
Severe/persistent bronchospasm:
• Nebulised salbutamol and ipratropium with oxygen
• Consider IV bolus and/or infusion of salbutamol or aminophylline
• Inhalational anaesthesia

C = Circulation
Give further fluid boluses and titrate to response:
Child 10 mL/kg per bolus
Adult 500–1000 mL per bolus
• Use glucose-free crystalloid (e.g. Hartmann's Solution, Plasma-Lyte®)
Large volumes may be required (e.g. 3–5 L in adults)
Place arterial cannula for continuous BP monitoring
Establish central venous access
IF REFRACTORY TO ADRENALINE INFUSION
Consider adding a second vasopressor in addition to adrenaline infusion:
• Noradrenaline, vasopressin or metaraminol
• In patients on beta-blockers, consider glucagon
Consider extracorporeal life support

Cardiac arrest – follow ALS ALGORITHM
• Start chest compressions early
• Use IV or IO adrenaline bolus (cardiac arrest protocol)
• Aggressive fluid resuscitation
• Consider prolonged resuscitation/extracorporeal CPR